Name:	MRS. LAXMI BHISE	**Age/Gender:**	29 years 4 months 16 days/Female
Referred By:	DR. TEJASHREE JAGTAP	**Client Name:**	N.A
Collection Date:	31-05-2026 00:00:00	**Report Release Date:**	02-06-2026 05:38:46

Sr.No	Investigation	Observed Value	Biological Ref. Interval	Unit

Triple Marker (HCG,AFP & E3)

Sr.No	Investigation	Observed Value	Biological Ref. Interval	Unit
1	Alpha Feto Protein (AFP) Serum, Method: CLIA	13.66		IU/mL
2	Beta HCG Serum, Method: CLIA	7495.0		mIU/mL
3	Estriol Serum, Method: CLIA	1.879		ng/ml

Remarks

Based on the demographic data, sonography report findings and biochemical marker analysis, the prenatal screening results are Negative for Trisomy 21 – Down syndrome (Biochemical Risk), Trisomy 18 and Neural tube defects.

Interpretation

1. Statistical risk factor calculation for Trisomy 21(Down's Syndrome), Trisomy 18 (Edward Syndrome) and Open Neural tube defect has been done using CE approved PRISCA 5 software.

2. The calculations are done using Indian medians, which are established in-house with database of more than 10000 patients and it is periodically updated.

3. All software may not give similar risk factor for the similar data.

4. This is a screening test and hence confirmation of screen positives is recommended.

5. It is advisable to ask for repeat calculations (not the test), in case history provided is not correct. For better reliability of results it is advised to carry out analysis between 15&17 weeks.

End Of Report

Scan For Report

* The analyte is not in the lab scope.
CRM No :15882413
Sample Recd. Time: 01-06-2026 18:28
Report Time: 02-06-2026 05:38
Patient Name: MRS. LAXMI BHISE
Patient ID: 15882413

Authorized Signatory
Dr. Sunil Kode
MBBS, DPB(Pathology)

Scan To Verify

Qualipath® Diagnostics
Quality you can trust
ISO 9001:2015 CERTIFIED

Better Care
Faster Results
Reliable Testing

Date of report:	**02/06/26**
Prisca	5.1.0.17

Dr.

Tejashree Jagtap

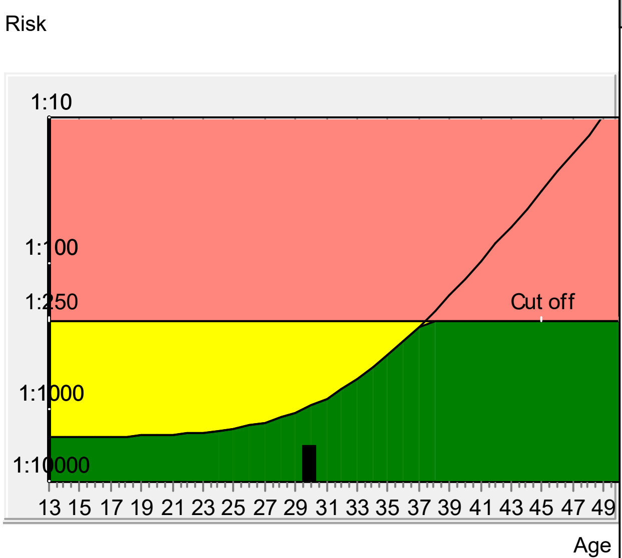
Patient data	
Name	MRS. LAXMI BHISE
Birthday	15/01/97
Age at delivery	29.8
Patient ID	15882413
Previous trisomy 21 pregnancies	unknown

Ultrasound data	
Gestational age	13 + 4
Method	CRL measurements
Crown rump length in mm	78.8
Date	20/05/26
Nuchal translucency MoM	0.52
Nuchal translucency	1.00 mm
Nasal bone	present
Sonographer	Sonologist
Qualifications in measuring NT	Sonologist

Correction factors			
Fetuses	1	diabetes	no
Weight	41.7	Origin	Asian
Smoker	no	IVF	unknown

Biochemical data		
Sample Date		31/05/26
Gestational age at sample date		15 + 1
Parameter	Value	Corr. MoMs
AFP	13.66 IU/mL	0.40
HCG	7495 mIU/mL	0.18
uE3	1.879 ng/mL	4.25

Risks at term	
Age risk	1:981
Trisomy 21 risk	1:5861
Combined trisomy 21 risk	<1:10000
Trisomy 18 risk	<1:10000



Trisomy 21

The calculated risk for Trisomy 21 (with nuchal translucency) is below the cut off, which indicates a low risk.
After the result of the Trisomy 21 test (with NT) it is expected that among more than 10000 women with the same data, there is one woman with a trisomy 21 pregnancy.
The AFP level is low.
The HCG level is low.
The calculated risk by PRISCA depends on the accuracy of the information provided by the referring physician.
Please note that risk calculations are statistical approaches and have no diagnostic value!
The patient combined risk presumes the NT measurement was done according to accepted guidelines (Prenat Diagn 18: 511-523 (1998)).
The laboratory can not be hold responsible for their impact on the risk assessment ! Calculated risks have no diagnostic value!

Trisomy 18

The calculated risk for trisomy 18 (with nuchal translucency) is < 1:10000, which represents a low risk.

Neural tube defects

The corrected MoM AFP (0.40) is located in the low risk area for neural tube defects.

below cut off | Below Cut Off, but above Age Risk | above cut off

ANNEXURE

Second trimester screening:

Multiple marker serum screening has become a standard tool used in obstetrical care to identify pregnancies that may have an increased risk for certain birth defects, including neural tube defects (NTD), trisomy 21 (Down syndrome), and trisomy 18 (Edwards syndrome). The screen is performed by measuring analytes in maternal serum that are produced by the fetus and the placenta. The analyte values along with maternal demographic information such as age, weight, gestational age, diabetic status, and race are used together in a mathematical model to derive a risk estimate. The laboratory establishes a specific cutoff for each condition, which classifies each screen as either screen-positive or screen-negative. A screen-positive result indicates that the value obtained exceeds the established cutoff. A positive screen does not provide a diagnosis, but indicates that further evaluation should be considered.

Alpha-fetoprotein (AFP)

The AFP concentration in maternal serum rises throughout pregnancy. If the fetus has an open NTD, AFP is thought to leak directly into the amniotic fluid causing unexpectedly high conc of AFP. Subsequently, the AFP reaches the maternal circulation, thus producing elevated serum levels. Other fetal abnormalities such as omphalocele, gastroschisis, congenital renal disease, esophageal atresia, and other fetal distress situations such as threatened abortion, and fetal demise also may show AFP elevations. Increased maternal serum AFP values also may be seen in multiple pregnancies and in unaffected singleton pregnancies in which the gestational age has been underestimated. Lower maternal serum AFP values have been associated with an increased risk for genetic conditions such as trisomy 21 (Down syndrome) and trisomy 18.

Reference Range	
(Males and nonpregnant females):	
<1 year	Upto 30 IU/ml
>1 year	Upto 8 IU/ml
Pregnant female:	
14 weeks of gestation	21.15 IU/ml
15 weeks of gestation	24.7 IU/ml
16 weeks of gestation	28.7 IU/ml
17 weeks of gestation	33.5 IU/ml
18 weeks of gestation	39.1 IU/ml
19 weeks of gestation	45.3 IU/ml
20 weeks of gestation	53.1 IU/ml
21 weeks of gestation	61.9 IU/ml

Beta-HCG (Total):

Human chorionic gonadotropin (hCG) is produced by the placenta and normally is only present during pregnancy. Some abnormal tissues, tumors, and cancers can also produce hCG, making the hCG test useful as a tumor marker. An increased level of hCG is seen with gestational trophoblastic disease and some germ cell tumors (benign and cancerous). Levels of hCG may also be elevated in other diseases such as liver, breast, lung, skin, and stomach cancers. Increased levels may also be seen in benign conditions such as cirrhosis, duodenal ulcer, inflammatory bowel disease, and marijuana use.

Reference Range	
Males and Non Pregnant Females	≤ 5.3 mIU/ml
Pregnant stage (as per Gestational age):	
1 - 3 weeks	16-4870 mIU/ml
3 - 4 weeks	10-31500 mIU/ml
4 - 5 weeks	60-82300 mIU/ml
5 - 6 weeks	3100-151000 mIU/ml
6 - 7 weeks	27300-233000 mIU/ml
7 -11 weeks	20900-291000 mIU/ml
11- 16 weeks	6140-103000 mIU/ml
>16 weeks	2700-81000 mIU/ml

3) Unconjugated Estriol (UE3)

Estriol, the principal circulatory estrogen hormone in the blood during pregnancy, is synthesized by the intact feto-placental unit. Estriol exists in maternal blood as a mixture of the unconjugated form and a number of conjugates. The half-life of unconjugated estriol in the maternal blood system is 20 to 30 minutes because the maternal liver quickly conjugates estriol to make it more water soluble for urinary excretion. Estriol levels increase during the course of pregnancy. Decreased unconjugated estriol has been shown to be a marker for trisomy 21 and trisomy 18. Low levels of estriol also have been associated with overestimation of gestation, pregnancy loss, Smith-Lemli-Opitz, and X-linked ichthyosis (placental sulfatase deficiency).

Reference Range	
Males and Non Pregnant Females	<2.0
Pregnant stage (as per Gestational age):	
16-18 weeks of gestation	0.3-2.3 ng/ml
18-32 weeks of gestation	5.3-18.3 ng/ml
32-36 weeks of gestation	5.2-28.1 ng/ml
37-38 weeks of gestation	8.0-38.0 ng/ml
39-40 weeks of gestation	7.2-28.9 ng/ml